

THE HUMAN INCIDENT RESPONSE PLAYBOOK

A two-page operational card. The reasoning, the evidence, and the sources are in the full framework.

IMMEDIATE DANGER — CALL 911. IN CRISIS — CALL OR TEXT 988. OUTSIDE THE US — FINDAHELPLINE.COM.

US veterans and service members: 988, then press 1.

If you are worried about someone right now, don't run a protocol. Use these, and stay with them.

1. THE FOUR ZONES

You're watching for **movement**, not for a state. A person who has always been quiet isn't a signal. A person who was talkative and now isn't, is.

- **Ready.** Recovering between exposures. Normal for them.
- **Reacting.** Distress that still resolves. Irritable, tired, short — but bounces back after a weekend.
- **Injured.** Not resolving on its own. The weekend doesn't fix it. Withdrawal, cynicism, changed sleep, work that used to matter no longer does.
- **Crisis.** Past what a colleague can hold. Route, don't handle.

2. FOUR INJURIES, FOUR FIXES

These are not severities of one thing. They're different injuries, and only one of them responds to rest.

Injury	What causes it	What actually helps
Burnout	Volume, tempo, no recovery gap, no control over the work	Rest, and a structural change to the load. Rest alone rebuilds nothing if they return to the same queue.
Secondary traumatic stress	Exposure to what happened to other people	Dose management and naming it. Time off does nothing.
Vicarious trauma	Years of accumulated exposure changing how they see the world	Longitudinal measurement. It won't show up in a single conversation.
Moral injury	Being party to something they believe was wrong, or stopped from doing what they believed was right	Acknowledgment of the specific cost. Not rest, not a lighter queue, and not relitigating the decision.

3. WHAT DRIVES EXPOSURE

Why one seat costs more than another: **content type** (what they have to look at), **decision weight** (who is affected by their call), **institutional friction** (being overruled, ignored, or unsupported), **cumulative dose** (how long, how often), **adversarial engagement** (a human being is trying to beat them), and **secrecy** (they can't tell anyone what they're carrying).

Secrecy is the one that breaks peer support. The injuries still apply. The remedies don't.

4. THE CHECK-IN — SEVEN C's

Check. Ask, observe, compare to their baseline — not to yours.

Coordinate. Work out who else needs to know, if anyone, and get their agreement first.

Cover. Get them out of the exposure. Reassign, reduce, rotate.

Calm. Slow it down. Don't problem-solve in the first five minutes.

Connect. Put them back in contact with people. Isolation is the accelerant.

Competence. Remind them they're good at this. Specifically, with an example.

Confidence. Restore their belief that it gets better, without promising it will.

Under classification, five of these still work. **Cover and Competence don't.** A peer outside the wall can affirm the person. They can't affirm the work.

5. WHAT TO SAY

- "You've seemed different the last couple of weeks. How are you doing, really?"
- "That case was a lot. What did it cost you?"
- "You don't have to tell me anything. I just wanted you to know I noticed."
- "What would make this week easier?"
- "I'm not asking as your lead right now."

6. WHAT NOT TO SAY

- "At least it wasn't..." — minimizing
- "You just need a vacation." — wrong fix for three of the four
- "We've all been there." — erases them
- "That's a you problem." — attributes it to character
- "You should talk to HR." — deflection, in the moment they came to you
- "Have you tried meditation?" — puts the fix on them
- "Everyone's slammed right now." — makes it a competition
- "You're stronger than this." — makes recovery a failure of will

7. WHEN TO ESCALATE

Hand off when any of these is true. You are not failing them by escalating. You are doing the one thing you can actually do.

- Any mention of harming themselves or anyone else. **Stop here, use the crisis numbers.**
- Orange with no improvement. Two check-ins is a reasonable point to stop and reassess.
- They ask for help you're not qualified to give.
- You're out of your depth, or you're carrying it home.
- The thing driving it is structural and above your authority.

Escalation is not surveillance. Route the person to support. Don't file a report on them, don't keep notes on a colleague, and tell them what you're doing and why.

8. WHERE TO ROUTE

- **Crisis:** 988 (call or text), 911, findahelpline.com.
- **Clinical:** their own clinician first. EAP is a route, not a guarantee — confidentiality varies by provider, contract and jurisdiction, and a formal referral often isn't confidential in the same way a self-initiated one is.
- **HR:** not a confidential clinical resource, and not a channel to avoid. It's the route to accommodations, leave, and schedule or workload changes. In the US those requests are what open an employer's obligations under the ADA. Clinical support is for the injury. HR is for the job.

9. LIMITS

This is a detection aid, not a safety system. People can be in serious trouble with none of these indicators present. Someone can hold a normal conversation, hit every deadline, answer honestly by their own reckoning, and still be nowhere near all right. Someone can also decline to tell you anything, and that's their right.

Running this well and still losing someone is possible. The protocol was never the thing holding the weight.

Before you use any of this: read the Stress First Aid manuals (free, National Center for PTSD) and consider Mental Health First Aid certification. This gives you vocabulary and a protocol. It can't give you judgment.

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